SACTCHECK TREATMENT ASSESSMENT

Ribociclib metastatic

NCCP 00525 | 6
SACTCheck v0.83.7

TUMOUR GROUP	INDICATION	COURSE
Breast	**Ribociclib with an aromatase inhibitor or fulvestrant for eligible HR-positive, HER2-negative locally advanced or metastatic**	**Advanced disease | q28d**

TREATMENT CONTEXT	ASSESSMENT ID	ASSESSMENT TIME
Not entered	**JSON-DEMO-001**	**17/10/2026, 03:21**

ENCODED CRITERIA RESULT

2 encoded criteria require clinical review

Review the highlighted protocol criteria and the current NCCP source before the final treatment decision.

ENTERED VALUES AND ENCODED CRITERIA

Standard assessment

DOMAIN	ENTERED VALUE	ENCODED CRITERION	RESULT
QTcF (ms)	490 ms	> 480 ms	**Criterion not met - review/hold pathway**
QTcF (ms)	490 ms	<= 500 ms	**Criterion not met - review/hold pathway**
Assessment stage	During treatment	= baseline	**Criterion met**
Assessment stage	During treatment	missing Not entered	**Criterion met**
QTcF (ms)	490 ms	>= 450 ms	**Criterion met**
QTcF (ms)	490 ms	<= 480 ms	**Criterion met**

COMPONENT-SPECIFIC PROTOCOL ACTIONS

Protocol-level guidance; verify prescribed dose/current level

Ribociclib: Withhold now - No dose now
Interrupt ribociclib for QTcF >480 ms. Correct reversible causes, repeat ECG and establish recurrence before any restart; do not calculate a restart dose from this reading alone. Current manufacturer SmPC distinguishes metastatic from early-breast restart and includes recurrent >500 ms discontinuation. NCCP v6 differs; reconcile with oncology/pharmacy before restart. page 5 | Table 5

UNASSESSED DOMAINS

ANC (x10^9/L), Febrile neutropenia / concurrent infection, Recurrent grade 3 neutropenia, Creatinine-clearance category, Child-Pugh class, Long-QT syndrome or significant QT/cardiac risk, Uncorrected relevant electrolyte abnormality, ECOG performance status + 34 more

Blank domains were not assumed normal.

CLINICIAN DECISION

Not documented
Not documented

CLINICAL DECISION SUPPORT - NOT TREATMENT CLEARANCE

SACTCheck applies encoded NCCP protocol criteria to clinician-entered information. This output does not constitute treatment clearance and does not replace complete clinical assessment, the current official NCCP protocol, prescribing information, local policy or professional judgement. The responsible oncology clinician retains responsibility for the final treatment decision.